

An Autistic Teenager in Your Clinic

For family doctors, nurses & mental-health professionals — caring for an autistic adolescent (roughly ages 12–18).

The one idea

Autism, through **monotropism**, means a teen’s attention locks into a few deep, intense “tunnels.” Puberty and adolescence pile on high-channel demands — shifting peer rules, identity, dating, body changes — on top of a system that finds rapid switching costly. This is the **peak age for anxiety, depression and burnout**, and many autistic teens (especially girls and marginalised genders) are **first identified only in crisis**. Treat the young person as the expert on their own experience.

What you may see — and what’s really going on

You may see...	What’s often happening
Exhausted, withdrawn, “can’t be bothered”; refusing school	Probable autistic burnout or overwhelm — distinct from depression and from laziness
Self-harm, suicidal thoughts, or crisis presentation	Real, elevated risk in this group; often the first sign of unmet need
Intense, consuming interests; resists giving them up	Interests are identity, regulation and recovery — not obsessions to limit
“Fine” in the appointment, collapses after	Heavy camouflaging ; the cost is paid later
Needs written contact, skips eye contact, long pauses	Single-channel communication + processing time, not rudeness

Try this

- **Offer the AASPIRE AHAT** pre-visit and a **written or online** appointment option; many teens communicate far better in writing.
- **Ask directly and separately** about mood, anxiety, burnout, sleep, eating, and suicidal thoughts — using concrete, literal questions.
- **Validate their interests** as legitimate; ask what they love and protect access to it.
- **Give processing time**; offer to send a written summary afterwards.
- **Involve them in decisions** and respect autonomy; build a predictable, low-sensory routine for follow-ups.

Avoid this

- Confusing **autistic burnout with depression** — treating it as depression (or “try harder”) can deepen it and raise suicide risk.
- Pushing the teen to mask, make eye contact, or “be more social” as a goal.
- Assuming a slow or unusual answer means they don’t understand or lack capacity.

When to get more support

Screen for suicidality directly — autistic teens are at elevated risk. Distinguish **autistic burnout** (exhaustion + skill loss + reduced tolerance to stimulus; needs *reduced demands, acceptance and permission to unmask*) from depression. Address puberty-related sensory and practical needs (menstruation, hygiene) concretely. Plan the **transition to adult services early**, with the young person's interests at the centre. Signpost autistic peer groups and mentors — high-quality friendship is powerfully protective.

If the teen is a girl or marginalised gender

Camouflaging peaks here and drives much of the missed diagnosis and mental-health cost. Presentation may look like “anxious, perfectionist, socially adept on the surface.” Ask specifically about **masking exhaustion** and internalising symptoms.

About this guide

*AI-assisted, derived from this project's research drafts — the **Monotropism** brief and the **Lifespan Practitioner & Health-Care guide** (Part 3.3 & 4). Uses identity-first language. **Informational — not medical advice or a diagnostic tool.** Fit it to the individual.*

Key sources. Murray, Lesser & Lawson (2005); Murray, F. (2023); Dwyer (2025); Raymaker et al. (2020, burnout); Pearson & Rose (2021, masking); Bradley et al. (2021, camouflaging); AASPIRE AHAT; Autism Awareness Australia (teen mental health). Full citations are in the source drafts.